Hospital Financial Model Analysis

Executive view

The base-case model projects a 300-bed hospital financed with $140.0 million of debt and $60.0 million of equity. Under the selected assumptions—78% occupancy, $1,100 revenue per occupied bed day, 3.0% annual revenue growth, and an 18.0% operating margin—the project generates positive EBITDA and supports scheduled debt service in every forecast year.

Year 1 revenue is $93.95 million, EBITDA is $16.91 million, and debt service is $12.95 million. The resulting Year 1 debt service coverage ratio (DSCR) is 1.31x. DSCR improves each year because EBITDA grows at 3.0% while annual principal payments remain fixed and interest expense declines as debt amortizes.

Operating forecast

Metric	Year 1	Year 5	Year 10	Year 25
Revenue	$93.95m	$105.74m	$122.58m	$190.98m
Operating expense	$77.04m	$86.71m	$100.52m	$156.61m
EBITDA / CFADS	$16.91m	$19.03m	$22.07m	$34.38m
EBITDA margin	18.0%	18.0%	18.0%	18.0%

Revenue growth is entirely assumption-driven. Starting revenue is calculated from occupied bed days: 300 beds multiplied by 78% occupancy, 365 days, and $1,100 per occupied bed day. The model then applies 3.0% annual growth. The operating margin remains constant at 18.0%, so EBITDA rises proportionally with revenue.

Debt-service capacity

The debt schedule uses straight-line amortization. Principal repayment is $5.60 million every year. Interest begins at $7.35 million in Year 1 and falls to $0.29 million in Year 25 as the outstanding balance is repaid.

Metric	Year 1	Year 5	Year 10	Year 15	Year 25
Beginning debt	$140.00m	$117.60m	$89.60m	$61.60m	$5.60m
Interest expense	$7.35m	$6.17m	$4.70m	$3.23m	$0.29m
Total debt service	$12.95m	$11.77m	$10.30m	$8.83m	$5.89m
DSCR	1.31x	1.62x	2.14x	2.90x	5.83x

The minimum modeled DSCR is 1.31x in Year 1, making the first years of operation the most important period for financing risk. Coverage improves to 2.14x by Year 10 and reaches 5.83x in

Year 25. The debt balance is fully repaid at the end of Year 25.

Scenario interpretation

The scenario tab provides downside, base, and upside operating cases:

Assumption	Downside	Base selected	Upside
Occupancy	65%	78%	85%
Revenue growth	1%	3%	5%
Operating margin	15%	18%	20%

The downside case presents the greatest pressure on debt capacity because it lowers both initial revenue through lower occupancy and future EBITDA through a lower margin and slower revenue growth. The base model's debt-service conclusion should therefore be tested by linking these scenario inputs to the full operating forecast and recalculating annual DSCR.

Key risks and model improvements

- The model equates CFADS with EBITDA. A lender-grade CFADS calculation would normally reflect cash taxes, maintenance capital expenditures, working-capital changes, and other required cash items.
- Occupancy, revenue per bed day, revenue growth, and operating margin are major drivers. Small adverse changes in these inputs can disproportionately affect the early-year DSCR.
- The model does not include depreciation, taxes, capital replacement, financing fees amortization, reserve accounts, or a refinancing case.
- The debt schedule assumes fixed straight-line principal amortization and a constant 5.25% rate. Actual financing may use a different repayment profile, floating rate, cash sweep, covenants, or reserve requirements.
- The operating forecast does not identify payer mix, reimbursement rates, patient volume by service line, staffing costs, or other hospital-specific operating drivers.

Overall assessment

On the model's stated base assumptions, the project can meet scheduled debt service from Year 1 onward, with coverage improving materially over time. The initial 1.31x DSCR provides a limited opening cushion relative to later years, so validating occupancy, reimbursement/yield, operating costs, and non-EBITDA cash-flow items is essential before relying on the model for an investment or credit decision.